Table 1	
Men/Women	9 (64%)/ 5 (36%)
Median age	55 years (40 – 68)
Primary tumor/relapse tumor	13 (93%) / 1 (7%)
Tumor Location	
Thigh	11 (79%)
Chest	1 (7%)
Arm	1 (7%)
Belly	1 (7%)
Histology	
Liposarcoma	7 (50%)
Fibroblastic sarcoma	3 (21.4%)
Pleomorfic sarcoma	2 (14.3%)
Desmoide tumor	1 (7.1%)
Sarcoma NOS	1 (7.1%)
Tumor grade	
G1	6 (42.8%)
G2	2 (14.3%)
G3	6 (42.8%)
Preoperative chemotherapy	
YES	4 (28.6%)
NO	10 (71.4%)

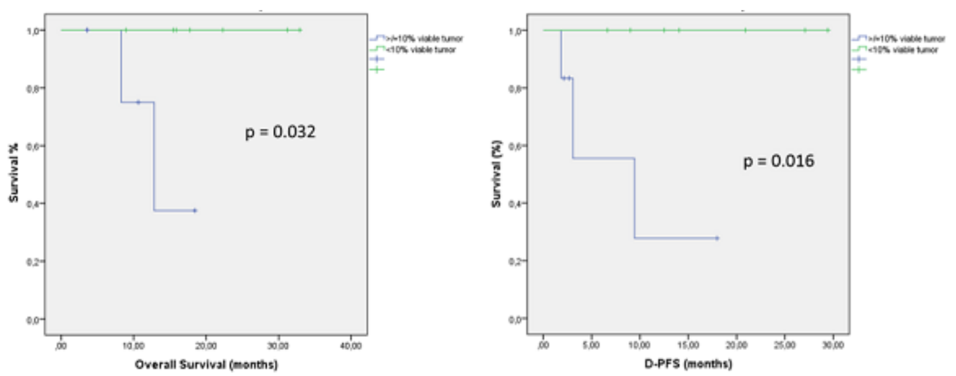
Table 2.	mPR	Pearson's chi-squared test
Age <55y vs. ≥55	43% vs 57%	0.593
Female vs. male	80% vs 33%	0.094
Thigh vs other location	54.5% vs 33%	0.515
Size x14cm vs ≥14cm	67% vs 45.5%	0.515
Liposarcoma vs. other	71.4% vs 28.6%	0.109
Grade 1-2 vs. 3	50% vs 50%	1
Preoperative chemotherapy: yes vs. no	100% vs 30%	0.018

Results

Eight patients (57%) reached mPR, being fully complete in 4 of them. On univariate analysis, mPR rates were higher for female patients, older patients, thigh location, smaller tumor size (<14cm), low grade tumors and liposarcoma histology although these differences were not statistically significant, probably due to low number of analyzed patients. However, addition of preoperative chemotherapy to radiation treatment associated higher probability of mPR. Complete analysis of relationship between mPR and clinical and epidemiological variables is detailed in table 2.

With a median follow-up of 13 months (7 – 19), 12 patients are alive, 2 patients died from distant tumor progression and no patient suffered from local relapse. Median overall survival (OS), local progression free survival (L-PFS) and distant progression free survival (D-PFS) times are not achieved. Actuarial 2-year OS and D-PFS are 79.5%, and 72.4% respectively. mPR is related to a statistically significant better OS (p=0.032) and better D-PFS (p=0.016). The absence of mPR was associated with higher risk of developing distant metastases (100% vs 36.4%, p = 0.051).

Treatment tolerance: 5 patients (35.7%) presented grade 1-2 acute skin toxicity after radiation treatment. Post-surgical complications: 5 patients (35.7%) had post-surgical seroma, 1 patient (7.1%) had neuropathic pain and 8 patients (57.1%) had wound complications. Five patients did not experience any degree of radiation or postsurgical side effects.



Conclusion

preoperative hypofractionated radiotherapy in 3-weeks is feasible, well tolerated and associates acceptable pathologic response rates. Achievement of mPR is related to better distant-metastases and overall survival. Addition of preoperative chemotherapy to radiation therapy could increase pathologic response rates and deserves further study searching for radio-enhancement synergies

PO-1429 First results of the Leiden-Holland Proton Therapy Center collaboration for uveal melanoma treatment

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Purpose or Objective

To evaluate clinical outcomes, including local tumour control, visual acuity, adverse events and eye preservation, of uveal melanoma patients treated at proton therapy centre HollandPTC in the Netherlands.

Materials and Methods